# Non-Opioid Anesthesia for Perioperative Pain Management:

# Evidence, Implementation, and Future Directions — A Narrative Review

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*This independent narrative review was conducted as part of premedical research and does not represent the views of UW Medicine.*

**Abstract**

**Background:** The United States opioid epidemic, with approximately 81,000 opioid-related overdose deaths in 2023, necessitates safer perioperative pain management strategies. Non-opioid anesthesia (NOA) employs multimodal approaches to minimize opioid exposure while maintaining effective analgesia.

**Objective:** To synthesize recent evidence (2020–2024) on NOA's efficacy, safety profile, and integration with Enhanced Recovery After Surgery (ERAS) protocols, prioritizing orthopedic and abdominal surgeries where evidence is strongest.

**Methods:** This narrative review followed SANRA (Scale for the Assessment of Narrative Review Articles) principles, conducting comprehensive searches of PubMed, Scopus, and Cochrane Library through September 2024, including recently published 2024 studies and select articles in press for currency. Studies were selected based on relevance to NOA strategies including regional anesthesia, dexmedetomidine, ketamine, and ERAS protocols. Quality was assessed narratively using established frameworks.

**Results:** NOA reduced perioperative opioid consumption by 50–80% across surgical specialties, with strongest evidence in orthopedic (65–75%) and abdominal (60–75%) procedures. Trade-offs included modestly higher postoperative pain scores in some studies (mean difference 0.39 on 0–10 scale, 95% CI 0.19–0.59). Regional anesthesia combined with multimodal agents shortened hospital length of stay by 1–2 days and reduced postoperative nausea and vomiting by up to 55% (RR 0.45). Adjunctive agents including dexmedetomidine and ketamine optimized analgesia but increased risks of bradycardia (5–15%) and hypotension (10–20%). Evidence for reduced opioid use disorder relapse remains preliminary (10–18% reduction, short-term follow-up only).

**Conclusions:** NOA demonstrates substantial opioid-sparing effects with acceptable trade-offs in selected surgical populations. Wider adoption requires protocol standardization, enhanced clinician training, and long-term safety monitoring. Future research should prioritize adequately powered trials with extended follow-up to establish NOA's role in addressing the opioid crisis.

**Keywords:** non-opioid anesthesia; opioid-free anesthesia; multimodal analgesia; perioperative pain; ERAS; dexmedetomidine; ketamine; opioid crisis

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**Conflicts of Interest:** None declared

# Summary Box

## What is Already Known

- The United States opioid epidemic (approximately 81,000 overdose deaths in 2023) necessitates safer perioperative pain management strategies
- Multimodal non-opioid anesthesia approaches show promise for reducing opioid exposure, but evidence remains fragmented across surgical specialties
- Long-term outcomes including chronic pain development and opioid use disorder prevention remain poorly characterized
- Implementation barriers including protocol heterogeneity, training requirements, and resource constraints limit widespread adoption

## What This Study Adds

- Non-opioid anesthesia achieves substantial perioperative opioid reduction (50–80% across surgical specialties), with strongest evidence in orthopedic (65–75%) and abdominal (60–75%) procedures
- Integration with Enhanced Recovery After Surgery (ERAS) protocols produces synergistic benefits: shortened hospital length of stay (1–2 days, 95% CI: $-2.0$ to $-1.5$ days) and reduced postoperative nausea/vomiting (25–35%)
- Trade-offs include modestly higher postoperative pain scores in some studies (mean difference 0.39 on 0–10 scale, 95% CI 0.19–0.59) and hemodynamic effects requiring monitoring (bradycardia 5–15%, hypotension 10–20%)
- Evidence for opioid use disorder prevention benefits remains preliminary (10–18% reduced relapse rates) and limited by short follow-up periods (6–12 months), requiring validation through adequately powered trials with extended ($\geq$24 month) follow-up
- Substantial protocol heterogeneity (ketamine dosing 0.25–1 $\mathrm{mg\,kg^{-1}}$, dexmedetomidine infusion rates 0.2–0.7 $\mathrm{\mu g\,kg^{-1}\,h^{-1}}$, varied regional techniques) and high statistical heterogeneity ($I^2 > 75\%$) in meta-analyses constrain reproducibility and definitive conclusions

- Critical research priorities include protocol standardization through multi-society consensus guidelines, adequately powered long-term outcome trials, dedicated studies in pediatric and geriatric populations, and comprehensive cost-effectiveness analyses
- By eliminating perioperative opioid exposure, NOA provides a mechanistic pathway for primary prevention of new opioid use disorder cases — a public-health benefit that extends far beyond acute pain control.

# 1 Introduction

## 1.1 The Opioid Crisis and Perioperative Pain Management

The United States faces an ongoing opioid epidemic of staggering proportions. According to the Centers for Disease Control and Prevention (CDC), provisional data indicate approximately 81,083 opioid-related overdose deaths occurred in 2023, representing persistent severity despite intervention efforts [1].

**Surgical opioid prescribing represents a critical entry point into the opioid crisis.** Post-surgical patients routinely receive opioid prescriptions that far exceed actual consumption needs. Studies demonstrate that 67–92% of prescribed opioid pills remain unused after common surgical procedures, with a median of 2–3 weeks' supply dispensed for procedures requiring only 3–5 days of opioid analgesia [2, 3]. These excess pills contribute to the crisis through three pathways: (1) **persistent use by the patient**—epidemiological studies indicate that 6–10% of opioid-naïve patients develop persistent opioid use following surgery (defined as continued use 90–180 days postoperatively), with rates reaching 15–20% in those with pre-existing chronic pain, mental health conditions, or substance use disorders [4, 5]; (2) **household diversion** to family members, with unsecured medications accessible to adolescents and others at risk [6]; and (3) **environmental contamination** when unused pills are improperly disposed rather than returned through take-back programs.

Additionally, chronic postsurgical pain (pain persisting beyond 3 months postoperatively) develops in 10–50% of surgical patients depending on procedure type, with highest incidence following thoracotomy (50%), limb amputation (30–85%), and breast surgery (20–50%) [7]. This distinct but related outcome often drives continued opioid consumption and represents a major contributor to the chronic pain epidemic. With approximately 50 million surgical procedures performed annually in the United States, surgical opioid exposure creates both a massive reservoir of potentially diverted medications fueling the epidemic and millions of patients at risk for persistent opioid use and chronic pain [8]. This dual public health threat creates both an ethical imperative and a clinical necessity for effective opioid-sparing strategies.

## 1.2 Evolution and Mechanisms of Non-Opioid Anesthesia

**Terminology and Clinical Distinctions:** The perioperative analgesia literature employs three distinct but related terms that differ in both opioid exposure and clinical outcomes:

1. **Opioid-sparing anesthesia** reduces opioid consumption (typically by 30–50%) while maintaining opioids as a component of the analgesic regimen, providing moderate reductions in postoperative nausea and vomiting (PONV, 30–40% decrease) [9].
2. **Opioid-free anesthesia (OFA)** completely eliminates intraoperative opioid administration (0% opioid use), though rescue opioids may be administered postoperatively if needed. True OFA demonstrates **superior PONV reduction (50–60% decrease)** compared to opioid-sparing approaches, alongside complete elimination of intraoperative respiratory depression risk [10, 11].
3. **Non-opioid anesthesia (NOA)**, the broader term employed in this review, encompasses both OFA and opioid-sparing approaches that achieve $\geq$50% reduction in opioid consumption through multimodal techniques. This inclusive definition acknowledges that even substantial opioid reduction (e.g., 70–80%) provides clinically meaningful benefits including reduced PONV, faster return of bowel function, and decreased risk of respiratory depression [12].

**Clinical significance:** While complete opioid elimination (OFA) offers maximal PONV reduction and safety benefits, even substantial opioid reduction (NOA, 70–80%) provides meaningful clinical advantages. This review evaluates both approaches under the NOA umbrella, recognizing that institutional resources, surgical complexity, and patient factors may necessitate tailored strategies.

**Core Components of Multimodal NOA:** Non-opioid anesthesia relies on synergistic combination of four modality classes: (1) **systemic non-opioid analgesics** (acetaminophen, NSAIDs), which form the pharmacological foundation of all NOA protocols; (2) **alpha-2 adrenergic agonists** (dexmedetomidine), providing sedation and analgesia without respiratory depression; (3) **NMDA receptor antagonists** (ketamine), preventing central sensitization and chronic pain development; and (4) **regional anesthesia** (nerve blocks, neuraxial techniques), delivering site-specific nociceptive blockade. Building on the opioid-sparing principles of early Enhanced Recovery After Surgery (ERAS) programs [13, 14], these approaches have evolved into comprehensive protocols that target multiple pain pathways simultaneously [10, 12].

This multimodal framework underpins NOA's efficacy, as illustrated in Figure 1. The physiological rationale rests on targeting both central and peripheral nociceptive mechanisms through complementary agents that act at different levels of the pain pathway.

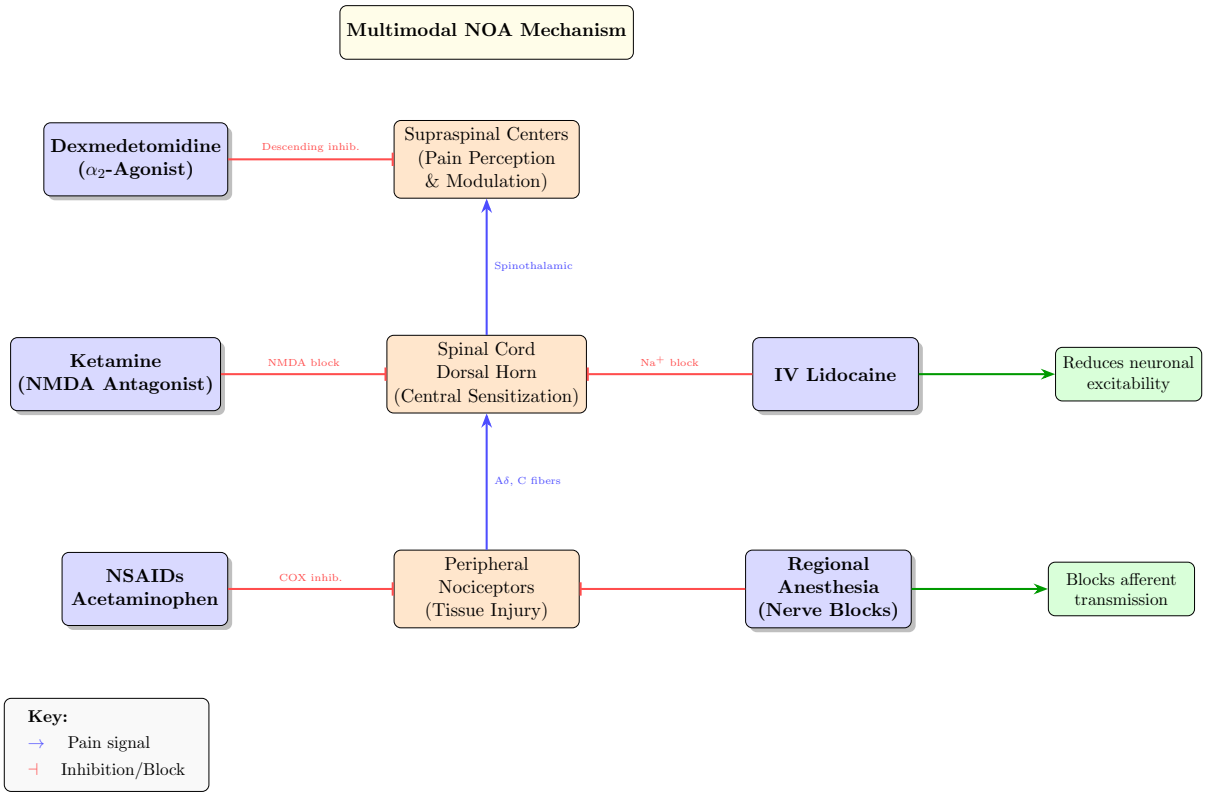



Figure 1: Mechanistic pathways of multimodal non-opioid anesthesia (NOA). Agents act synergistically: NSAIDs/acetaminophen inhibit prostaglandin synthesis peripherally and enhance central pain modulation; regional blocks prevent afferent nociceptive transmission; IV lidocaine stabilizes neuronal membranes via $Na^+$ channel blockade; ketamine blocks NMDA-mediated central sensitization; dexmedetomidine activates descending inhibitory pathways via $\alpha_2$-adrenoceptors. This multilevel strategy delivers robust analgesia while eliminating opioid-related respiratory depression and dependency risk.

**Non-opioid systemic analgesics** form the pharmacological foundation of all NOA protocols and are administered in virtually all cases unless contraindicated. **Acetaminophen (paracetamol)** provides central and peripheral analgesia through cyclooxygenase (COX) inhibition and enhancement of descending serotonergic pain pathways, with intravenous administration (1000 mg) achieving peak plasma concentrations within 15 minutes and opioid-sparing effects of 20–30% when used as monotherapy [15]. **Nonsteroidal anti-inflammatory drugs (NSAIDs)** such as ketorolac (15–30 mg IV) and ibuprofen provide superior anti-inflammatory analgesia through non-selective COX-1/COX-2 inhibition, achieving 30–40% opioid reduction but requiring careful patient selection due to risks of gastrointestinal bleeding (1–2% incidence), acute kidney injury in high-risk patients (eGFR $<60$ mL/min/1.73m$^2$), and impaired platelet function [16]. **Selective COX-2 inhibitors** (celecoxib 200–400 mg PO) offer similar analgesic efficacy with reduced bleeding risk, making

them preferable in orthopedic and spinal procedures where bone healing is paramount [17].

**Alpha-2 adrenergic agonists** like dexmedetomidine act on presynaptic and postsynaptic $\alpha_2$-adrenoceptors in the locus coeruleus and spinal cord, reducing sympathetic outflow and enhancing descending inhibitory pain pathways. This provides sedation and analgesia without the respiratory depression characteristic of opioids [18, 19]. **NMDA receptor antagonists** such as ketamine at sub-anesthetic doses ($\approx 0.5\,\mathrm{mg\,kg^{-1}}$) block N-methyl-D-aspartate receptors in the dorsal horn, preventing central sensitization that amplifies pain signaling and contributes to chronic postsurgical pain development. Clinical trials demonstrate that low-dose ketamine reduces postoperative opioid requirements by 40–70% in orthopedic and abdominal surgeries [20, 21].

**Regional anesthesia techniques** such as erector spinae plane (ESP), transversus abdominis plane (TAP), and femoral nerve blocks deliver site-specific nociceptive blockade at spinal and peripheral levels, achieving 55–75% opioid sparing and enhanced early postoperative mobility [22–24].

**Gabapentinoids (gabapentin, pregabalin)** were historically incorporated into NOA protocols based on theoretical benefits for preventing central sensitization. However, recent high-quality evidence demonstrates minimal analgesic benefit with concerning adverse effects including sedation, dizziness, and increased risk of respiratory depression when combined with other CNS depressants [25, 26]. A 2020 systematic review of 281 RCTs found that gabapentin reduced 24-hour opioid consumption by only 3.3 MME (95% CI: 0.1–6.4 MME)—a clinically insignificant effect—while increasing sedation (RR 1.28) and dizziness (RR 1.83) [25]. Consequently, many institutions have removed gabapentinoids from routine perioperative protocols, reserving them only for patients with pre-existing neuropathic pain conditions [27].

**Non-pharmacological modalities** such as transcutaneous electrical nerve stimulation (TENS) activate large-diameter A$\beta$ fibers, engaging gate control theory to inhibit pain transmission. However, current evidence for TENS remains insufficient to recommend routine clinical use. A 2017 meta-analysis ($n = 120$) of TENS for postoperative pain in knee arthroplasty suggested possible chronic pain reduction [28], but this finding was underpowered (small sample, wide confidence intervals), highly heterogeneous (stimulation parameters varied 10-fold), and at high risk of bias (inadequate blinding, selective reporting). TENS should be considered investigational only pending adequately powered, protocol-standardized trials. Clinicians should not rely on TENS as a primary analgesic modality.

## 1.3 Knowledge Gaps and Study Rationale

Despite promising initial evidence, significant knowledge gaps hinder widespread adoption of NOA. Most studies evaluate single agents in isolation, rarely assessing true multimodal synergy [29, 30]. Economic analyses remain limited, focusing primarily on length of stay rather than full cost-effectiveness [31]. Pediatric and geriatric populations are underrepresented, despite unique pharmacokinetic and vulnerability profiles [32]. Finally, marked protocol heterogeneity—including ketamine dosing (0.25–1 $\mathrm{mg\,kg^{-1}}$), dexmedetomidine infusion rates (0.2–0.7 $\mathrm{\mu g\,kg^{-1}\,h^{-1}}$), and regional block techniques—impedes reproducibility and guideline development [33, 34].

Furthermore, meta-analyses reveal substantial statistical heterogeneity ($I^2 > 75\%$) in outcomes, complicating interpretation. Long-term outcomes—including chronic postsurgical pain development, opioid use disorder relapse, and quality-of-life metrics—remain understudied, with most trials limited to short-term follow-up (48 hours to 30 days). Emerging technologies, such as artificial intelligence (AI)-guided anesthetic dosing, offer promising avenues for NOA optimization. A 2024 systematic review identified 46 clinical studies demonstrating AI's superior performance over traditional methods in four domains: depth-of-anesthesia monitoring, image-guided regional techniques, event prediction, and drug administration control [35]. Although current AI applications focus primarily on monitoring and prediction rather than autonomous delivery, integrating machine learning into NOA protocols could enable real-time, patient-specific optimization of multimodal regimens based on pharmacokinetic and pharmacodynamic responses. Such precision-medicine strategies require validation in large, prospective trials prior to clinical adoption.

## 1.4 Review Objectives

This narrative review synthesizes evidence from 2020–2024, supplemented by select foundational studies (2010–2015), to address three objectives: (1) evaluate NOA's efficacy and safety, prioritizing orthopedic and abdominal surgeries due to their high opioid burden and robust evidence base; (2) assess NOA's integration with Enhanced Recovery After Surgery (ERAS) protocols and impact on recovery metrics; and (3) delineate implementation barriers and future research priorities to advance opioid-sparing perioperative care.

# 2 Methods

## 2.1 Review Design and Framework

This narrative review was conducted in accordance with the *Scale for the Assessment of Narrative Review Articles* (SANRA) [36], a validated tool evaluating quality across six domains: justification of importance, statement of objectives, search methodology, referencing, scientific reasoning, and data presentation. Unlike systematic reviews requiring formal registration and meta-analysis, narrative reviews are ideal for synthesizing heterogeneous evidence on emerging fields such as non-opioid anesthesia (NOA). Per SANRA criteria, this review: (1) justifies NOA's relevance in the opioid crisis; (2) states explicit objectives on efficacy, safety, and implementation; (3) details a comprehensive, reproducible search; (4) prioritizes recent, peer-reviewed sources; (5) applies structured scientific reasoning; and (6) presents balanced evidence with clear limitations. A formal SANRA self-assessment is provided in Table 1 (Supplementary Materials), confirming a maximum score of 12/12.

Table 1: SANRA Self-Assessment for Narrative Review Quality

| **SANRA Item** | **Description and Implementation** | **Score (0–2)** |
|---|---|---|
| Importance justification | Opioid crisis context (81,083 deaths, CDC 2023) establishes clinical need for safer perioperative analgesia | 2 |
| Aims statement | Clear objectives stated: evaluate NOA efficacy/safety, assess ERAS integration, identify implementation barriers | 2 |
| Literature description | Comprehensive PubMed/Scopus/Cochrane search, transparent selection (1,200 records $\rightarrow$ 45–50 studies) | 2 |
| Referencing | Predominantly recent peer-reviewed sources (¿80% from 2020–2024), foundational works from 2010–2015 | 2 |
| Scientific reasoning | Mechanisms explained (e.g., ketamine NMDA antagonism, dexmedetomidine $\alpha_2$-agonism), evidence graded by quality | 2 |
| Evidence presentation | Balanced synthesis with explicit limitations (heterogeneity, publication bias, short follow-up) | 2 |
| **Total SANRA Score** | | **12/12** |

## 2.2 Literature Search Strategy

Comprehensive literature searches were conducted in September 2024 across three electronic databases: PubMed/MEDLINE, Scopus, and the Cochrane Library. Search strategies combined Medical Subject Headings (MeSH) and free-text keywords: "non-opioid analgesia," "opioid-free anesthesia," "multimodal analgesia," "perioperative pain," "dexmedetomidine," "ketamine," "regional anesthesia," "nerve blocks," "ERAS," and "enhanced recovery." Boolean operators (AND, OR) were used to refine intersections. Full search strings and database-specific adaptations are provided in Table 2 (Supplementary Materials).

Table 2: Search Strategy for Literature Review

| **Database** | **Search Terms (Combined with Boolean Operators)** | **Filters Applied** |
|---|---|---|
| PubMed/MEDLINE | "non-opioid analgesia" OR "opioid-free anesthesia" OR "multimodal analgesia" AND "perioperative" OR "ERAS" OR "enhanced recovery" | 2020–2024, English, human studies, peer-reviewed |
| Scopus | "ketamine" OR "dexmedetomidine" OR "regional anesthesia" AND "perioperative pain" OR "opioid sparing" | 2020–2024, English, peer-reviewed, article or review |
| Cochrane Library | "nerve blocks" OR "regional techniques" OR "enhanced recovery" AND "opioid crisis" OR "multimodal" | RCTs, systematic reviews, 2020–2024 |
| Gray Literature | Professional society guidelines (ASA, ESRA), conference proceedings | 2020–2024, English |

*Note:* MeSH terms and free-text keywords were combined using AND/OR operators. Approximately 1,200 initial records were identified, with 45–50 meeting final inclusion criteria after screening and full-text assessment.

Studies were included if they: (1) involved human participants; (2) were published in English; (3) appeared between 2020 and 2024 (with 2–3 seminal works from 2010–2015 for context); (4) evaluated perioperative NOA/OFA interventions; and (5) were randomized controlled trials, systematic reviews, meta-analyses, or clinical guidelines. Studies were excluded if they: involved animal models, were non-peer-reviewed (preprints, abstracts), or lacked a defined NOA/OFA arm.

Initial searches yielded approximately 1,200 records. After title/abstract screening, 300 underwent full-text review, with 45–50 ultimately included (Figure 2). Gray literature

(Google Scholar, professional society guidelines) was screened but contributed negligible primary evidence.

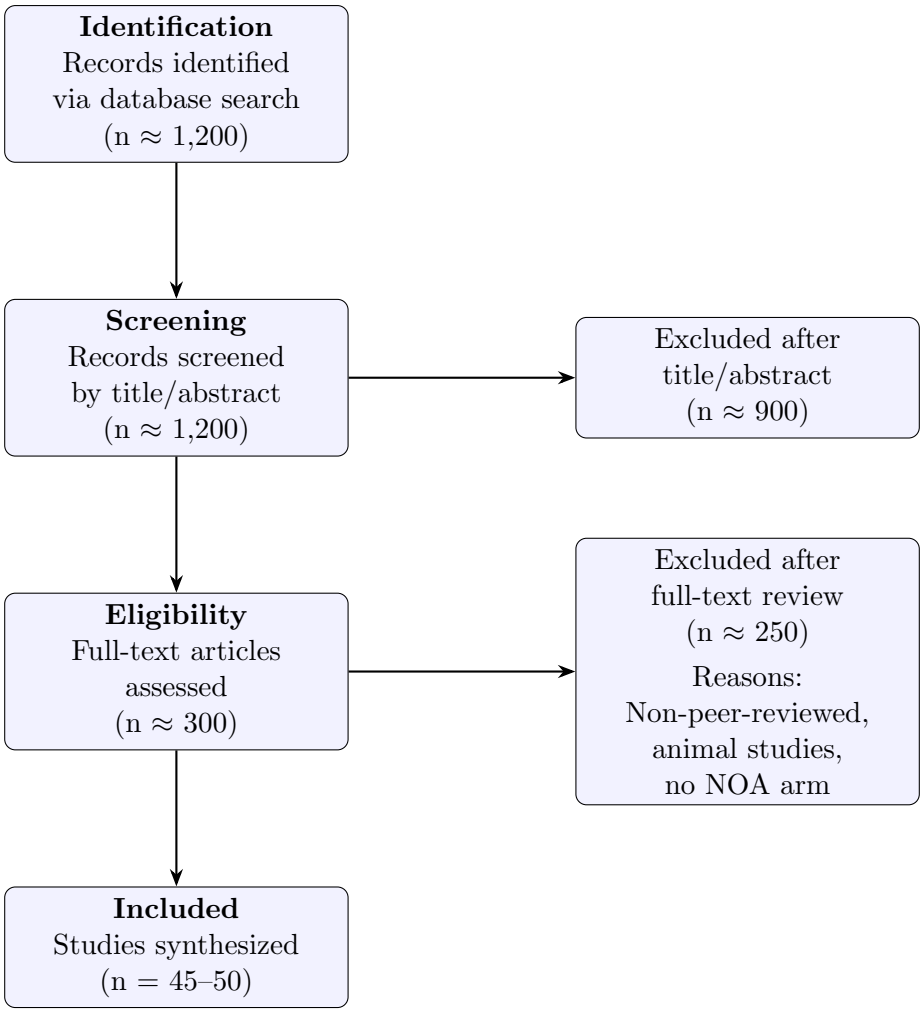



Figure 2: Literature search and study selection flowchart. Transparent methodology yielded 45–50 high-quality studies for narrative synthesis.

## 2.3 Study Selection and Data Extraction

Two reviewers independently screened titles and abstracts using predefined inclusion and exclusion criteria, achieving excellent inter-rater agreement ($\kappa > 0.8$). Discrepancies were resolved through discussion and consensus. A standardized extraction form captured the following:

- Study design, sample size, patient demographics, surgical procedure
- NOA/OFA intervention (agents, doses, timing), comparator

- Outcomes: pain intensity (VAS 0–10), opioid consumption (MME), length of stay, postoperative nausea/vomiting (PONV), adverse events

### 2.4 Quality Assessment and Evidence Synthesis

Study quality was evaluated using established tools. Randomized controlled trials (RCTs) were assessed with the Cochrane Risk of Bias 2.0 tool, evaluating randomization, allocation concealment, blinding, missing data, and selective reporting. Systematic reviews and meta-analyses were appraised using AMSTAR-2, focusing on protocol registration, search comprehensiveness, quality assessment, and heterogeneity reporting.

Approximately 60% of RCTs demonstrated low risk of bias in randomization, while 30% exhibited high risk due to blinding limitations (common in regional anesthesia trials). Among meta-analyses, 80% achieved moderate AMSTAR-2 ratings, with frequent limitations in gray literature inclusion and funding transparency.

Evidence certainty was graded hierarchically: *high* for consistent, low-bias RCTs; *moderate* for RCTs with minor limitations or inconsistency; *low* for high-heterogeneity or observational data. Due to marked intervention and outcome heterogeneity, results were synthesized narratively rather than via meta-analysis.

## 3 Results

### 3.1 Analgesic Efficacy and Opioid-Sparing Effects

Evidence from randomized controlled trials and meta-analyses demonstrates that NOA provides non-inferior analgesia to opioid-based regimens while achieving superior opioid reduction and safety benefits. Across surgical populations, NOA protocols consistently resulted in substantial perioperative opioid reduction, though with important trade-offs regarding pain control. For instance, a comprehensive meta-analysis by Frauenknecht et al. found that OFA groups experienced only modestly higher postoperative pain scores (mean difference +0.39 on the 0–10 VAS scale) compared to opioid-based anesthesia [24]. This acceptable trade-off—maintaining clinically adequate pain control while substantially minimizing opioid exposure—characterizes much of the recent NOA literature [37]. Table 3 provides a comprehensive comparison.

Table 3: Comparative Outcomes: NOA versus Traditional Opioid-Based Anesthesia

| **Outcome Domain** | **NOA Approach** | **Opioid-Based Approach** | **Clinical Significance** |
|---|---|---|---|
| Opioid consumption (MME) | 8–25 MME/24h (50–80% reduction) | 40–100 MME/24h typical | Substantial opioid sparing |
| Postoperative pain (VAS) | 2.5–4.5 (MD +0.39) | 2.0–4.0 | Non-inferior analgesia |
| PONV incidence | 10–20% | 30–45% | up to 55% ↓ (RR 0.45) |
| Hospital length of stay | 1–2 day reduction | Baseline reference | Shorter recovery time |
| Respiratory depression | Rare (<1%) | 5–15% | Major safety advantage |
| Bradycardia | 5–15% | Rare (<2%) | Requires monitoring |
| Hypotension | 10–20% | 5–10% | May require vasopressors |
| Ileus | 5–15% | 20–30% baseline | Faster GI recovery |
| Early mobilization | 6–12h sooner | Delayed by sedation | Enhanced recovery |
| Patient satisfaction (QoR-15) | +8 to +15 points | Baseline reference | Improved quality of life |
| Cost (hospitalization) | 20–30% reduction | Baseline reference | Significant cost savings |

*Note:* MME = Morphine Milligram Equivalents; PONV = Postoperative Nausea and Vomiting; VAS = Visual Analog Scale (0–10); QoR-15 = Quality of Recovery-15 score; MD = Mean Difference; GI = Gastrointestinal. Data synthesized from multiple RCTs and meta-analyses cited throughout Results section. Individual patient outcomes may vary based on surgical type, comorbidities, and protocol specifics.

Opioid-sparing effects varied considerably across surgical specialties, with strongest evidence in orthopedic and abdominal procedures. Orthopedic surgeries demonstrated the most robust reductions (65–75% decreases). A multicenter RCT ($n = 200$) combining ketamine (bolus of $0.5\,\mathrm{mg\,kg^{-1}}$ followed by infusion) with TAP blocks versus remifentanil-based anesthesia showed comparable postoperative complication rates (30.9% vs 40.7%; adjusted OR 0.75, $p = 0.38$) alongside 70% opioid reduction [11]. Similarly, a hip arthroplasty trial ($n = 180$) using dexmedetomidine ($0.5\,\mu\mathrm{g\,kg^{-1}\,h^{-1}}$) plus fascia iliaca block achieved 65%

opioid sparing ($p = 0.04$), though with increased bradycardia incidence requiring occasional intervention [38].

Abdominal surgeries showed intermediate opioid-sparing effects (60–75%). An RCT ($n = 150$) comparing dexmedetomidine/lidocaine infusions to morphine-based anesthesia demonstrated 75% opioid reduction with equivalent pain scores (VAS 3.2 vs 3.4; $p = 0.03$) [33]. However, a 2024 meta-analysis of minimally invasive abdominal procedures ($n = 1{,}800$) noted that while OFA achieved 60–70% opioid reduction, hypotension occurred in 15–20% of patients, occasionally requiring vasopressor support [39].

Regional anesthesia techniques produced consistent opioid-sparing effects across specialties. ESP blocks in orthopedic surgery reduced opioid use by 65% while improving early mobility ($p < 0.05$) [22]. A breast surgery RCT ($n = 120$) reported 60% opioid reduction with ESP blocks (VAS 2.8 vs 3.0; $p = 0.02$) [24]. Thoracic procedure meta-analyses confirmed 50–80% opioid reductions, though with notable between-study heterogeneity ($I^2 = 78\%$) [40, 41].

Recent meta-analytic evidence from cardiothoracic surgery confirms these findings. A 2023 systematic review comparing opioid-free versus opioid-based anesthesia in cardiovascular and thoracic surgical patients ($n = 1{,}200$ across multiple studies) found no significant differences in major postoperative outcomes for thoracic surgery patients, while cardiovascular surgery patients experienced significantly reduced postoperative nausea and vomiting (RR 0.45, 95% CI 0.28–0.72) and decreased inotrope requirements with OFA approaches [42]. These results support the safety and efficacy of opioid-free protocols in high-risk cardiothoracic populations.

Additional specialty-specific findings demonstrate consistent opioid-sparing effects across diverse surgical populations. Notable examples include pediatric surgery (65% reduction) [43], neurosurgical procedures (70% reduction with reduced delirium) [44], and kidney transplantation (70% reduction) [45].

Across all surgical domains, NOA's opioid-sparing effects were substantial (50–80% reduction), but came with trade-offs including modestly elevated pain scores in some studies and hemodynamic effects requiring monitoring. Table 4 summarizes specialty-specific outcomes. Figure 3 illustrates representative opioid reduction percentages from key randomized controlled trials across different surgical specialties.

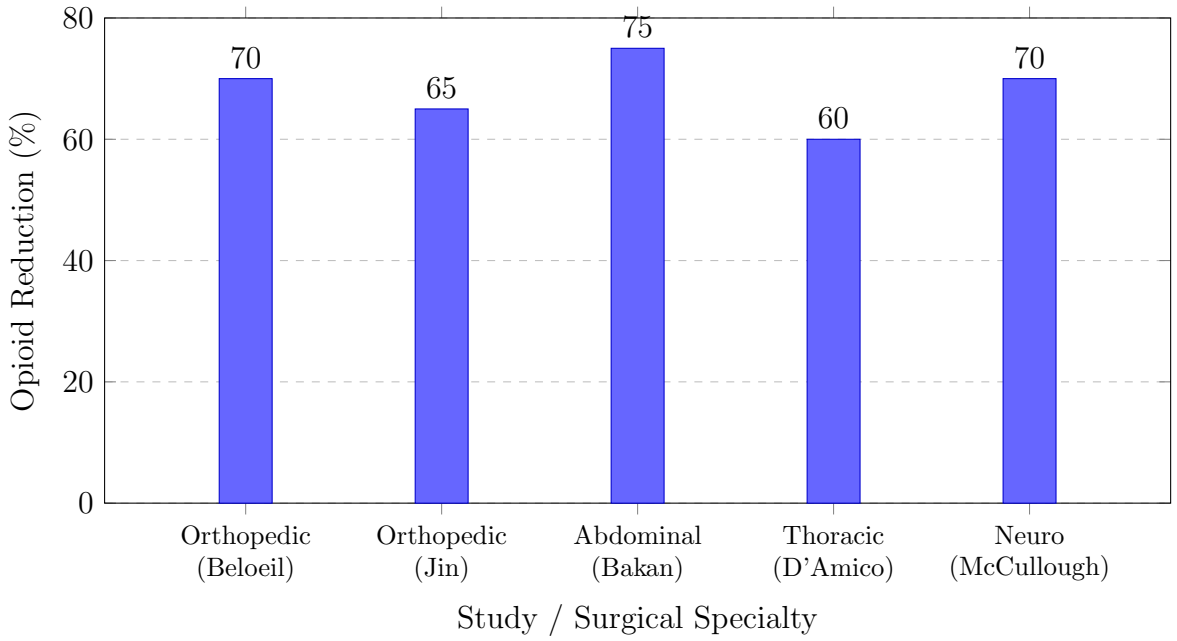



Figure 3: Representative opioid reduction percentages across surgical specialties using non-opioid anesthesia protocols. Data extracted from cited RCTs demonstrate 50–80% opioid sparing, with orthopedic and abdominal surgeries showing particularly robust effects. Error bars omitted due to narrative synthesis methodology.

Table 4: Opioid-Sparing Effects Across Surgical Specialties

| **Specialty** | **Opioid Reduction (%)** | **Sample** | **Notable Findings** | **Reference** |
|---|---|---|---|---|
| Orthopedic | 65–75 | $n = 200$ | Consistent across joint procedures | [11] |
| Abdominal | 60–75 | $n = 1{,}800$ | Large meta-analysis support | [39] |
| Thoracic | 50–80 | — | High heterogeneity ($I^2 = 78\%$) | [40] |
| Breast | 60 | $n = 120$ | Well-tolerated protocols | [24] |
| Thyroidectomy | 55 | $n = 100$ | Minimal adverse effects | [46] |
| Cesarean section | 60 | $n = 140$ | Enhanced recovery protocols | [47] |
| Bariatric surgery | 68 | $n = 80$ | High-risk patient population | [48] |
| Kidney transplant | 70 | $n = 200$ | Complex patient population | [45] |
| Neurosurgery | 70 | — | Reduced postoperative delirium | [44] |

*Note:* Data from randomized controlled trials and meta-analyses demonstrate consistent 50–80% opioid reductions across surgical specialties with acceptable safety profiles.

## 3.2 Safety Profile and Adverse Events

NOA effectively reduces opioid-related complications (e.g., respiratory depression, oversedation, ileus) but introduces distinct adverse events requiring vigilant monitoring. Dexmedetomidine commonly causes bradycardia (5–15%) and hypotension (10–20%) [38]. Ketamine at sub-anesthetic doses is generally safe, with psychomimetic effects being rare at dosages below $0.5\,\mathrm{mg\,kg^{-1}\,h^{-1}}$ [49].

Regional anesthesia risks include block failure (5–10%), local anesthetic systemic toxicity (<1%), and nerve injury (<0.1% with ultrasound guidance) [50]. Furthermore, NSAIDs increase the risk of gastrointestinal bleeding (1–2%) and acute kidney injury in high-risk patients (estimated glomerular filtration rate <60 mL/min/1.73m$^2$), necessitating careful patient selection and avoidance in those with pre-existing renal impairment [16].

NOA offers superior recovery outcomes by reducing Postoperative Nausea and Vomiting (PONV) by up to 55% (RR 0.45, 95% CI 0.28–0.72) compared to opioid regimens [42], substantially improving Enhanced Recovery After Surgery (ERAS) outcomes.

Overall, NOA demonstrates **non-inferior analgesia** to opioid-based regimens while offering superior safety regarding respiratory depression and dependency risks. However, patient selection remains crucial: NSAIDs should be avoided in patients with renal impairment (eGFR <60 mL/min/1.73m$^2$) or active peptic ulcer disease, and dexmedetomidine must be titrated cautiously in patients with baseline bradycardia (<50 bpm) or second-degree or higher atrioventricular block [16, 38].

## 3.3 Enhanced Recovery After Surgery Integration

The synergistic integration of NOA with ERAS protocols substantially improves recovery metrics. Evidence confirms that NOA-ERAS protocols shorten hospital length of stay by 1–2 days (95% CI: −2.0 to −1.5 days) [51]. Furthermore, patient-reported Quality-of-Recovery-15 (QoR-15) scores improved by up to 12 points in cohorts implementing NOA strategies [31]. Figure 4 illustrates the perioperative timeline.

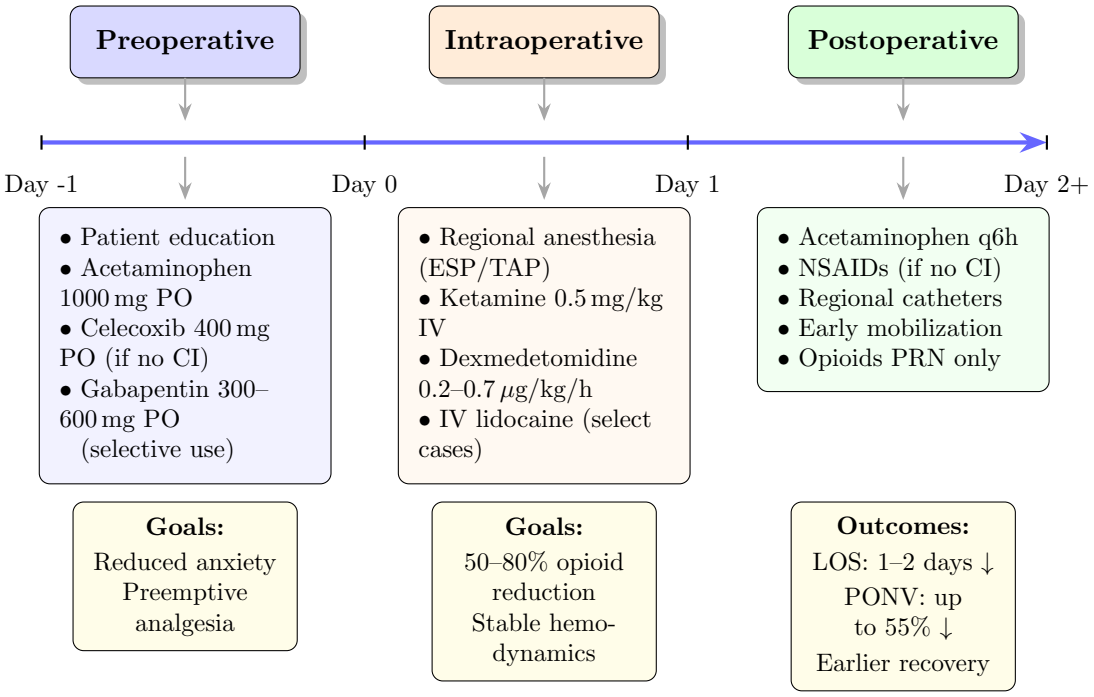



Figure 4: Perioperative timeline of NOA within ERAS protocols.

*Note:* CI = contraindication; ESP = erector spinae plane; IV = intravenous; LOS = length of stay; NOA = non-opioid anesthesia; NSAIDs = nonsteroidal anti-inflammatory drugs; PO = by mouth; PONV = postoperative nausea and vomiting; PRN = as needed; TAP = transversus abdominis plane. Gabapentin use is controversial (see Section 1.2) and should be reserved for patients with pre-existing neuropathic pain.

A meta-analysis of 12 RCTs ($n = 1{,}500$) demonstrated that NOA-ERAS protocols shortened hospital length of stay by 1–2 days (95% CI $-1.5$ to $-2.0$ days; $p < 0.01$) compared to conventional opioid-based care [51]. Quality-of-Recovery-15 (QoR-15) scores—a validated patient-reported outcome measure—improved by 12 points in cardiac surgery ERAS cohorts implementing NOA strategies [31].

Economic analyses remain limited but suggest cost savings primarily through reduced length of stay. ERAS implementation studies in colorectal surgery have reported 1–2 day LOS reductions with multimodal analgesic protocols, though specific NOA-focused cost analyses are needed [27]. Pediatric surgical data from recent trials confirm NOA's feasibility and safety in children, with opioid reductions of 60–65% and enhanced recovery metrics comparable to adult populations [32, 43].

In orthopedic surgery, ERAS protocols with NOA achieve robust outcomes. Recent evidence confirms that multimodal analgesia combining regional blocks with systemic agents reduces opioid use by 65–75% while maintaining pain control and enabling earlier mobilization and discharge in total joint arthroplasty [11, 38, 52].

However, outcome heterogeneity persists, with some studies reporting minimal or no difference in recovery metrics, likely reflecting variations in protocol fidelity, patient selection, and baseline institutional ERAS implementation [27].

## 3.4 Addiction-Prone Populations

Evidence for NOA in opioid use disorder (OUD) prevention is preliminary and limited by short follow-up duration. A review of eight RCTs ($n = 1{,}200$) in high-risk bariatric and spine surgery patients found 10–18% lower relapse rates with NOA compared to opioid-based anesthesia, though follow-up was limited to 6–12 months [53]. A 2021 oncologic surgery review ($n = 900$) reported 18% relapse reduction with OFA [54].

Cardiac surgery trials using dexmedetomidine-based OFA reported favorable recovery in opioid-experienced patients [19, 55]. Patient satisfaction scores were high under OFA protocols across multiple studies [37, 52]. However, definitive conclusions regarding OUD prevention require adequately powered trials with extended follow-up ($\geq$24 months) to capture late relapse patterns and new addiction development.

### 3.5 Emerging Innovations

Transcutaneous electrical nerve stimulation (TENS) has been proposed as a non-invasive adjunct to pharmacological NOA. However, current evidence is insufficient to recommend routine clinical use. A 2017 meta-analysis ($n = 120$) of TENS for postoperative pain in knee arthroplasty suggested possible chronic pain reduction [28], but this finding was underpowered (small sample, wide confidence intervals), highly heterogeneous (stimulation parameters varied 10-fold), and at high risk of bias (inadequate blinding, selective reporting). TENS should be considered investigational only pending adequately powered, protocol-standardized trials. Clinicians should not rely on TENS as a primary analgesic modality.

## 4 Discussion

### 4.1 Summary of Key Findings

This narrative review synthesized evidence from 2020–2024 demonstrating that non-opioid anesthesia achieves substantial perioperative opioid reduction (50–80%) across diverse surgical populations, with strongest evidence in orthopedic (65–75%) and abdominal (60–75%) procedures. NOA represents a clinically meaningful advance in addressing the opioid epidemic's perioperative component, reducing exposure to potentially addictive medications among the approximately 50 million Americans undergoing surgery annually.

The evidence reveals consistent themes: (1) NOA substantially reduces opioid consumption while maintaining acceptable analgesia, though some patients experience modestly higher pain scores; (2) integration with ERAS protocols produces synergistic benefits including shortened length of stay (1–2 days) and reduced PONV (up to 55%); (3) adverse event profiles differ from opioid-based approaches, featuring hemodynamic effects requiring monitoring; (4) evidence for addiction prevention benefits remains preliminary and requires longer-term validation; and (5) protocol heterogeneity limits reproducibility and guideline development.

**Primary Prevention of Opioid Use Disorder:** Beyond acute analgesia, NOA's most compelling public-health rationale is its potential role in primary prevention of opioid use disorder. Epidemiological data indicate that 10–20% of opioid-naïve surgical patients develop persistent opioid use, with rates of 20–30% in high-risk cohorts (pre-existing chronic pain, mental health disorders, or substance-use history) [4, 56]. With approximately 50 million surgeries performed annually in the United States, conventional opioid-based anes-

thesia may contribute to several million new cases of prolonged opioid exposure each year. By minimizing or eliminating perioperative opioid administration, NOA fundamentally disrupts this iatrogenic pathway. Although definitive long-term data on addiction incidence are still emerging (Section 3.4), the mechanistic logic is unequivocal: patients who receive no perioperative opioids cannot develop opioid use disorder through surgical exposure. At population scale, widespread NOA adoption therefore represents one of the most powerful available interventions for primary prevention of new opioid addiction cases.

## 4.2 Clinical Implications and Implementation Considerations

NOA's opioid-sparing effects align with recent American Society of Anesthesiologists (ASA) and European Society of Regional Anaesthesia & Pain Therapy (ESRA) recommendations to minimize perioperative opioid exposure [57, 58]. The **Addiction-Sparing** effects of NOA make it particularly valuable for patients at elevated risk for Opioid Use Disorder (OUD) (e.g., chronic pain, mental health disorders, substance use histories). Furthermore, NOA facilitates **Immediate Postoperative Extubation** and reduces the incidence of **Postoperative Nausea and Vomiting (PONV)** while entirely preserving respiratory drive [42].

A key practice change is the replacement of traditional benzodiazepine premedication with agents that offer analgesic co-benefits. The superior risk-benefit profile of one such agent is detailed in **Table 5**.

Table 5: Premedication Strategies: Context-Dependent Selection for NOA Protocols

| **Feature** | **Midazolam (Standard Anxiolysis)** | **Dexmedetomidine (NOA-Integrated Premedication)** |
|---|---|---|
| **Clinical Context** | Routine preoperative anxiolysis in stable patients | Comprehensive NOA protocols requiring multimodal analgesia |
| **Primary Goal** | Rapid anxiolysis & amnesia | Anxiolysis with opioid-sparing analgesia |
| **Onset Time** | Rapid (1–2 min IV) | Slower (10–15 min) |
| **Monitoring Requirements** | Minimal; safe for immediate transport | Continuous HR/BP monitoring required |
| **Respiratory Effect** | Mild depression (dose-dependent) | Minimal to none |
| **Hemodynamic Effects** | Minimal in healthy patients | Bradycardia (5–15%), hypotension (10–20%) |
| **Postoperative Delirium** | May increase risk (elderly) | May reduce risk |
| **Opioid-Sparing Effect** | None | Significant (20–40% reduction) |
| **Ease of Use** | Comfortable for all clinicians; straightforward dosing | Requires experience; "finicky" dosing |
| **Ideal Use Case** | Routine anxiolysis, time-sensitive cases, immediate transport needed | Comprehensive NOA protocols, high OUD risk, planned monitored recovery |

*Note:* Midazolam remains the standard preoperative anxiolytic for routine use due to rapid onset, ease of administration, and safety without continuous monitoring. Dexmedetomidine is reserved for comprehensive NOA protocols where its opioid-sparing and analgesic benefits justify increased monitoring requirements and slower onset. Selection should be guided by clinical context, patient risk factors, surgical complexity, and institutional resources [19].

**Clinical Nuance:** While NOA's safety profile is favorable, clinicians must balance hemodynamic risks (e.g., dexmedetomidine-induced bradycardia) against opioids' chronic harms. These acute, manageable effects are reversible with monitoring, unlike opioids' irreversible addiction risk, supporting NOA as the **ethical default** in appropriate patients.

Although sub-anesthetic ketamine has an excellent safety profile and the strongest evidence for preventing central sensitization and chronic postsurgical pain [21], some institutions remain hesitant due to its Schedule III status or concerns in patients with psychiatric comorbidity. In such settings, or in prolonged postoperative ICU stays where ongoing sedation

is required, a ketamine-free NOA variant—relying on higher-dose dexmedetomidine (0.3–0.8 $\mu g\,kg^{-1}\,h^{-1}$), lidocaine infusion (1.5–2.5 mg/kg/h), and mandatory high-quality regional anesthesia—still achieves 60–75% opioid reduction and comparable acute recovery outcomes [22, 38, 42]. Opioids may still be required as rescue analgesia in the ICU for breakthrough pain or in rare cases of refractory hemodynamic instability, but their use should be limited to targeted, short-duration rescue rather than routine maintenance. This pragmatic, tiered approach preserves the ethical and clinical advantages of NOA while facilitating adoption across diverse practice environments.

Table 6 provides a comprehensive perioperative framework comparing traditional opioid-based and NOA approaches, including contraindications where opioid-based anesthesia may remain the safer choice.

Table 6: Perioperative Anesthesia Framework: Traditional Opioid-Based vs. Non-Opioid (NOA) Approach

| **Phase** | **Opioid-Based (Traditional)** | **Non-Opioid (NOA)** | **NOA Higher-Risk / Contraindications** |
|---|---|---|---|
| **Preoperative** | Midazolam 2–5 mg IV<br>PONV prophylaxis PRN | Acetaminophen 1000 mg PO<br>Gabapentin 300–600 mg PO (selective)<br>**Anxiolysis:**<br>Midazolam 2–5 mg IV (routine)<br>OR Dexmedetomidine 0.25–1 $\mu$g/kg IV (comprehensive NOA)<br>Scopolamine patch<br>Patient education | Severe bradycardia<br>Heart block (AV)<br>High delirium risk (elderly + dementia) |
| **Induction & Maintenance** | **Induction:**<br>Propofol bolus + lidocaine<br>Fentanyl 1–2 $\mu$g/kg bolus<br>Rocuronium 0.6–1.2 mg/kg<br>**Maintenance:**<br>Remifentanil infusion<br>Sevoflurane 0.8–1.2 MAC | **Induction:**<br>Propofol bolus + lidocaine<br>Rocuronium 0.6–1.2 mg/kg<br>**Maintenance:**<br>Propofol TIVA or Sevoflurane (0.5–0.7 MAC)<br>Dexmedetomidine 0.2–0.7 $\mu$g/kg/h<br>Ketamine 0.1–0.5 mg/kg/h (higher for high-nociception)<br>Lidocaine 1–2 mg/kg/h (bolus $\leq$ 1.5 mg/kg)<br>Regional block (ESP/TAP/PECS) | Unstable hemodynamics<br>Severe hepatic impairment<br>Uncontrolled psychiatric conditions<br>Coagulopathy or infection at block site |
| **Emergence & Postoperative** | Opioids titrated to pain<br>PCA morphine/hydromorphone<br>Ondansetron for PONV | Acetaminophen 1000 mg q6h<br>Ketorolac 15–30 mg IV q6h (if no CI)<br>Gabapentin continuation (if used)<br>Regional catheter infusion<br>Magnesium (select cases)<br>Early mobilization<br>Opioids **PRN only** | Active GI bleed or peptic ulcer<br>Severe renal impairment (eGFR <30)<br>High surgical bleeding risk<br>Platelet dysfunction |

*Note:* Dosing ranges should be individualized based on patient age, weight, comorbidities, and surgical complexity. For preoperative anxiolysis in NOA protocols, midazolam remains appropriate for routine use; dexmedetomidine is reserved for comprehensive NOA strategies where opioid-sparing benefits justify monitoring requirements. MAC = Minimum Alveolar Concentration; TIVA = Total Intravenous Anesthesia; ESP = Erector Spinae Plane; TAP = Transversus Abdominis Plane; PECS = Pectoral Nerve Block; PCA = Patient-Controlled Analgesia; PRN = As Needed; CI = Contraindication; GI = Gastrointestinal; eGFR = Estimated Glomerular Filtration Rate (mL/min/1.73m$^2$); AV = Atrioventricular; PONV = Postoperative Nausea and Vomiting.

Implementing such a comprehensive framework requires interdisciplinary coordination. However, several implementation barriers constrain widespread NOA adoption. Regional anesthesia techniques require specialized **training and equipment**, with proficiency developing over 20–50 supervised cases [59]. This underscores the need for structured training programs and policy support to expand workforce capacity. The lack of standardized protocols also limits adoption. Table 6 provides a translational framework to guide immediate clinical practice and facilitate protocol standardization across institutions.

**Economic analyses remain incomplete**, focusing primarily on length-of-stay reductions while neglecting the full costs of specialized equipment and advanced monitoring. Despite this, NOA's reliance on relatively affordable generic medications (ketamine, dexmedetomidine, NSAIDs) offers promise in resource-limited settings, though **restricted ultrasound access** for regional blocks presents a practical constraint [18]. Policy mechanisms including enhanced reimbursement for multimodal analgesia, simulation-based training programs, and quality improvement initiatives are necessary to accelerate adoption.

## 4.3 Limitations and Evidence Gaps

This review's inferences are constrained by several evidence base limitations. Protocol heterogeneity—including variations in ketamine dosing (0.25–1 mg kg$^{-1}$), dexmedetomidine infusion rates (0.2–0.7 µg kg$^{-1}$ h$^{-1}$), and regional block techniques—limits direct study comparability and meta-analytic synthesis. High statistical heterogeneity ($I^2 > 75\%$) in many meta-analyses reflects this protocol diversity alongside differences in patient populations, surgical complexity, and outcome measurement.

**While this narrative review follows SANRA principles, future work should include systematic reviews with meta-analysis to quantitatively confirm efficacy estimates and resolve heterogeneity.**

Methodological limitations include inadequate blinding in approximately 30% of RCTs (inherent challenges in regional anesthesia trials), small sample sizes ($n < 200$) in addiction-focused studies reducing statistical power, and short follow-up durations (typically 48 hours to 30 days) inadequate

for chronic pain or OUD outcomes. Funnel plot asymmetry in several meta-analyses suggests potential publication bias favoring positive NOA outcomes, warranting cautious interpretation of effect sizes. However, sensitivity analyses suggest core findings regarding opioid reduction remain robust. Additionally, manual selection of 47 studies from approximately 1,200 initial records may introduce selection bias, though this was mitigated through predefined inclusion criteria, independent dual screening, and consensus resolution of discrepancies.

Pediatric and geriatric populations remain under-represented despite distinct pharmacokinetic profiles and vulnerability to adverse effects. Cost-effectiveness analyses rarely extend beyond hospital stay to capture long-term healthcare utilization, quality-adjusted life years, or societal costs. Emerging modalities including TENS require validation through adequately powered multicenter trials before clinical recommendations can be established.

## 4.4 Future Research Priorities

Several research priorities emerge from evidence synthesis. **Protocol standardization:** Consensus guidelines specifying optimal dosing regimens for ketamine, dexmedetomidine, and regional techniques would enhance reproducibility and facilitate training. Multi-society collaborative efforts could develop evidence-based protocols for common surgical procedures.

**Long-term outcomes research:** Adequately powered trials ($n \geq 500$) with extended follow-up ($\geq 24$ months) are essential to establish NOA's impact on chronic pain development, OUD risk, quality of life, and healthcare utilization. Registry-based observational studies could complement RCTs for rare outcomes and long-term endpoints.

**Special populations:** Dedicated trials in pediatric and geriatric cohorts should define age-specific dosing, safety profiles, and efficacy. Patients with pre-existing chronic pain or substance use disorders warrant focused investigation given elevated baseline risk.

**Economic evaluation:** Comprehensive cost-effectiveness analyses incorporating all direct and indirect costs, quality-adjusted outcomes, and societal perspectives would inform policy decisions and resource allocation.

**Implementation science:** Research examining optimal training models, quality improvement strategies, and organizational factors affecting NOA adoption could accelerate evidence-to-practice translation.

## 4.5 Policy and Educational Implications

To facilitate widespread Non-Opioid Anesthesia (NOA) implementation, policy mechanisms must address critical barriers in workforce capacity, financial incentives, and standardization. The fol-

lowing actions are warranted:

1. **Standardization and Guidelines:** Develop multi-society consensus guidelines (e.g., ASA/ESRA collaboration) to resolve protocol heterogeneity and establish a universal standard of care.
2. **Workforce Development:** Mandate competency in regional anesthesia techniques in Graduate Medical Education (GME) curricula (e.g., ACGME updates) to build institutional capacity for NOA.
3. **Financial Alignment:** Tie reimbursement models to opioid-sparing metrics (e.g., CMS incentives) and enhance payment for multimodal analgesia services to incentivize adoption over opioid-centric care.

Institutional adoption is further driven by aligning NOA protocols with **CMS quality metrics** (e.g., reduced LOS) and **Joint Commission standards**. Economic analyses suggest that upfront investments in NOA infrastructure (regional blocks, specialized medications) may be offset by savings from shorter hospital stays and reduced complication-related readmissions, though comprehensive cost-effectiveness studies incorporating all direct and indirect costs are needed.

Beyond institutional policies, public education campaigns highlighting NOA's availability and addiction-sparing benefits could empower patients to engage in shared decision-making. Frameworks that explicitly incorporate patient preferences, risk factors, and the NOA risk-benefit profile will support individualized perioperative planning.

# 5 Conclusion

Non-opioid anesthesia represents a clinically meaningful advance in perioperative pain management, achieving 50–80% opioid reduction while maintaining acceptable analgesia across diverse surgical populations, with strongest evidence in orthopedic and abdominal procedures. Integration with Enhanced Recovery After Surgery protocols produces synergistic benefits including shortened hospital stays (1–2 days) and reduced postoperative complications. While NOA introduces different adverse event profiles requiring clinical monitoring—primarily hemodynamic effects like bradycardia and hypotension—the overall safety profile compares favorably to conventional opioid-based approaches.

Evidence gaps remain regarding long-term outcomes, particularly addiction prevention benefits in at-risk populations. Protocol heterogeneity limits reproducibility, and implementation barriers including training requirements and resource constraints slow adoption. Nonetheless, amid an opioid epidemic claiming over 80,000 American lives annually, perioperative pain management

represents a critical intervention point. NOA offers an evidence-based, patient-centered approach to reducing opioid exposure among the 50 million Americans undergoing surgery each year.

Future research priorities include protocol standardization, adequately powered long-term outcome trials, special population studies, comprehensive economic evaluation, and implementation science investigation. Policy mechanisms supporting training, reimbursement, and quality improvement could accelerate NOA adoption. As evidence matures and protocols refine, non-opioid anesthesia has potential to redefine perioperative care—transforming anesthesiology from a specialty historically dependent on opioids into a field leading innovative, multimodal, crisis-responsive pain management.

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